

House File 852

H-1291

1 Amend House File 852 as follows:

2 1. By striking everything after the enacting clause and
3 inserting:

4 <DIVISION I

5 PHARMACY BENEFITS MANAGERS

6 Section 1. Section 510B.1, Code 2025, is amended by adding
7 the following new subsections:

8 NEW SUBSECTION. 11A. "*National average drug acquisition*
9 *cost*" means the monthly survey of retail pharmacies conducted
10 by the federal centers for Medicare and Medicaid services
11 to determine average acquisition cost for Medicaid covered
12 outpatient drugs.

13 NEW SUBSECTION. 11B. "*Pass-through pricing*" means a
14 model of prescription drug pricing in which payments made
15 by a third-party payor to a pharmacy benefits manager for
16 prescription drugs are equivalent to the payments the pharmacy
17 benefits manager makes to the dispensing pharmacy or dispensing
18 health care provider for the prescription drugs, including any
19 professional dispensing fee.

20 NEW SUBSECTION. 16A. "*Pharmacy chain*" means an entity that
21 has twenty or more pharmacies under common ownership or control
22 located in at least twenty or more states.

23 NEW SUBSECTION. 21A. "*Retail pharmacy*" means a pharmacy
24 that is not a pharmacy chain or a publicly traded entity, and
25 that does not exclusively provide mail order dispensing of
26 prescription drugs.

27 NEW SUBSECTION. 21B. "*Specialty drug*" means a drug used
28 to treat chronic and complex, or rare medical conditions and
29 that requires special handling or administration, provider care
30 coordination, or patient education that cannot be provided by a
31 nonspecialty pharmacy or pharmacist.

32 NEW SUBSECTION. 22A. "*Wholesale acquisition cost*" means the
33 same as defined in 42 U.S.C. §1395w-3a(c)(6)(B).

34 Sec. 2. Section 510B.4, Code 2025, is amended by adding the
35 following new subsection:

1 NEW SUBSECTION. 4. A pharmacy benefits manager, health
2 carrier, health benefit plan, or third-party payor shall not
3 discriminate against a pharmacy or a pharmacist with respect to
4 participation, referral, reimbursement of a covered service, or
5 indemnification if a pharmacist is acting within the scope of
6 the pharmacist's license, as permitted under state law, and the
7 pharmacy is operating in compliance with all applicable laws
8 and rules.

9 Sec. 3. NEW SECTION. 510B.4B Prohibited conduct — pharmacy
10 rights.

11 1. A pharmacy benefits manager shall not do any of the
12 following:

13 a. If a pharmacy or pharmacist has agreed to participate
14 in a covered person's health benefit plan, prohibit or limit
15 the covered person from selecting a pharmacy or pharmacist of
16 the covered person's choice, or impose a monetary advantage
17 or penalty that would affect a covered person's choice.

18 A monetary advantage or penalty includes a copayment or
19 coinsurance variation, a reduction in reimbursement for
20 services, a promotion of one participating pharmacy over
21 another, or comparing the reimbursement rates of a pharmacy
22 against mail order pharmacy reimbursement rates.

23 b. Deny a pharmacy or pharmacist the right to participate as
24 a contract provider under a health benefit plan if the pharmacy
25 or pharmacist agrees to provide pharmacy services that meet
26 the terms and requirements of the health benefit plan and the
27 pharmacy or pharmacist agrees to the terms of reimbursement
28 set forth by the third-party payor for similarly classified
29 pharmacies.

30 c. Impose upon a pharmacy or pharmacist, as a condition
31 of participation in a third-party payor network, any course
32 of study, accreditation, certification, or credentialing that
33 is inconsistent with, more stringent than, or in addition to
34 state requirements for licensure or certification, and the
35 administrative rules adopted by the board of pharmacy.

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1 *d.* Unreasonably designate a prescription drug as a
2 specialty drug to prevent a covered person from accessing
3 the prescription drug, or limiting a covered person's access
4 to the prescription drug, from a pharmacy or pharmacist that
5 is within the health carrier's network. A covered person or
6 pharmacy harmed by an alleged violation of this paragraph may
7 file a complaint with the commissioner, and the commissioner
8 shall, in consultation with the board of pharmacy, make a
9 determination as to whether the covered prescription drug meets
10 the definition of a specialty drug.

11 *e.* Require a covered person, as a condition of payment
12 or reimbursement, to purchase pharmacy services, including
13 prescription drugs, exclusively through a mail order pharmacy.

14 *f.* Impose upon a covered person a copayment, reimbursement
15 amount, number of days of a prescription drug supply for
16 which reimbursement will be allowed, or any other payment
17 or condition relating to purchasing pharmacy services from
18 a pharmacy that is more costly or restrictive than would be
19 imposed upon the covered person if such pharmacy services were
20 purchased from a mail order pharmacy, or any other pharmacy
21 that can provide the same pharmacy services for the same cost
22 and copayment as a mail order service.

23 2. *a.* If a third-party payor providing reimbursement to
24 covered persons for prescription drugs restricts pharmacy
25 participation, the third-party payor shall notify, in writing,
26 all pharmacies within the geographical coverage area of the
27 health benefit plan restriction, and offer the pharmacies
28 the opportunity to participate in the health benefit plan at
29 least sixty days prior to the effective date of the health
30 benefit plan restriction. All pharmacies in the geographical
31 coverage area of the health benefit plan shall be eligible to
32 participate under identical reimbursement terms for providing
33 pharmacy services and prescription drugs.

34 *b.* The third-party payor shall inform covered persons of
35 the names and locations of all pharmacies participating in

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1 the health benefit plan as providers of pharmacy services and
2 prescription drugs.

3 c. A participating pharmacy shall be entitled to announce to
4 the pharmacy's customers that the pharmacy participates in the
5 health benefit plan.

6 3. The commissioner shall not certify a pharmacy benefits
7 manager or license an insurance producer that is not in
8 compliance with this section.

9 4. A covered person or pharmacy injured by a violation
10 of this section may maintain a cause of action to enjoin the
11 continuation of the violation.

12 Sec. 4. Section 510B.8, Code 2025, is amended by adding the
13 following new subsections:

14 NEW SUBSECTION. 3. A pharmacy benefits manager shall not
15 impose different cost-sharing or additional fees on a covered
16 person based on the pharmacy at which the covered person fills
17 a prescription drug order.

18 NEW SUBSECTION. 4. For the purpose of reducing premiums,
19 one hundred percent of all rebates received by a pharmacy
20 benefits manager shall be passed through to the health carrier,
21 or to the employee plan sponsor as permitted by the federal
22 Employee Retirement Income Security Act of 1974, 29 U.S.C.
23 §1001, et seq.

24 NEW SUBSECTION. 5. A pharmacy benefits manager shall
25 include any amount paid by a covered person, or on behalf of
26 a covered person, when calculating the covered person's total
27 contribution toward the covered person's cost-sharing.

28 NEW SUBSECTION. 6. Any amount paid by a covered person for
29 a prescription drug shall be applied to any deductible imposed
30 on the covered person by the covered person's health benefit
31 plan in accordance with the health benefit plan's coverage
32 documents.

33 NEW SUBSECTION. 7. If a covered person's policy, contract,
34 or plan providing for third-party payment or prepayment of
35 health or medical expenses qualifies as a high-deductible

1 health plan under section 223 of the Internal Revenue Code,
2 and a copayment, coinsurance, or deductible paid by the
3 covered person as a cost-sharing requirement under this chapter
4 would result in the covered person becoming ineligible for a
5 health savings account associated with the covered person's
6 high-deductible health plan, subsection 5 shall apply only
7 after the covered person satisfies the covered person's minimum
8 deductible, except for items or services determined to be
9 preventive care under section 223(c)(2)(C) of the Internal
10 Revenue Code.

11 Sec. 5. Section 510B.8B, Code 2025, is amended to read as
12 follows:

13 **510B.8B Pharmacy benefits manager affiliates managers —**
14 **~~reimbursement~~ reimbursements.**

15 1. A pharmacy benefits manager shall not reimburse any
16 pharmacy located in the state in an amount less than the amount
17 that the pharmacy benefits manager reimburses a pharmacy
18 benefits manager affiliate for dispensing the same prescription
19 drug as dispensed by the pharmacy. ~~The reimbursement amount~~
20 ~~shall be calculated on a per unit basis based on the same~~
21 ~~generic product identifier or generic code number.~~

22 2. A pharmacy benefits manager shall not reimburse any
23 retail pharmacy located in the state in an amount less than the
24 most recently published national average drug acquisition cost
25 for a prescription drug on the date that the prescription drug
26 is administered or dispensed. If the most recently published
27 national average drug acquisition cost for the prescription
28 drug is unavailable on the date that the prescription drug is
29 administered or dispensed, a pharmacy benefits manager shall
30 not reimburse any retail pharmacy located in the state in
31 an amount less than the wholesale acquisition cost for the
32 prescription drug on the date that the prescription drug is
33 administered or dispensed.

34 3. In addition to the reimbursement required under
35 subsection 2, a pharmacy benefits manager shall reimburse the

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1 retail pharmacy or pharmacist a professional dispensing fee in
2 the amount of ten dollars and sixty-eight cents.

3 4. a. A pharmacy benefits manager shall submit a quarterly
4 report to the commissioner of all drugs reimbursed at ten
5 percent or more below the national average drug acquisition
6 cost, and all drugs reimbursed at ten percent or more above the
7 national average drug acquisition cost, for each prescription
8 drug appearing on the national average drug acquisition cost
9 list on the day the prescription drug was dispensed.

10 b. For each prescription drug included in the report, a
11 pharmacy benefits manager shall include all of the following
12 information:

13 (1) The month the prescription drug was dispensed.

14 (2) The quantity of the prescription drug dispensed.

15 (3) The amount the pharmacy was reimbursed.

16 (4) If the dispensing pharmacy was an affiliate of the
17 pharmacy benefits manager.

18 (5) If the prescription drug was dispensed pursuant to a
19 government health plan.

20 (6) The average national drug acquisition cost for the month
21 the prescription drug was dispensed.

22 c. The report shall exclude drugs dispensed pursuant to 42
23 U.S.C. §256b.

24 d. A copy of the report shall be published on the pharmacy
25 benefits manager's public internet site for twenty-four months
26 after the date the report is submitted to the commission.

27 5. This section shall not apply to a pharmacy that operates
28 in a state-owned facility.

29 **Sec. 6. NEW SECTION. 510B.8D Pharmacy benefits manager**
30 **contracts.**

31 **1. All contracts executed, amended, adjusted, or renewed**
32 **on or after July 1, 2025, that apply to prescription drug**
33 **benefits on or after January 1, 2026, between a pharmacy**
34 **benefits manager and a third-party payor, or between a person**
35 **and a third-party payor, shall include all of the following**

1 requirements:

2 *a.* The pharmacy benefits manager shall use pass-through
3 pricing.

4 *b.* Payments received by a pharmacy benefits manager for
5 services provided by the pharmacy benefits manager to a
6 third-party payor or to a pharmacy shall be used or distributed
7 pursuant to the pharmacy benefits manager's contract with
8 the third-party payor or with the pharmacy, or as otherwise
9 required by law.

10 2. Unless otherwise prohibited by law, subsection 1 shall
11 supersede any contractual terms to the contrary in any contract
12 executed, amended, adjusted, or renewed on or after July 1,
13 2025, that applies to prescription drug benefits on or after
14 January 1, 2026, between a pharmacy benefits manager and a
15 third-party payor, or between a person and a third-party payor.

16 Sec. 7. NEW SECTION. **510B.8E Appeals and disputes.**

17 1. A pharmacy benefits manager shall provide a reasonable
18 process to allow a pharmacy to appeal any matter.

19 2. The appeals process must include all of the following:

20 *a.* A dedicated telephone number at which a pharmacy may
21 contact the pharmacy benefits manager and speak directly with
22 an individual who is involved with the appeals process.

23 *b.* A dedicated electronic mail address or internet site for
24 the purpose of submitting an appeal directly to the pharmacy
25 benefits manager.

26 *c.* A period of no less than thirty business days after the
27 date of a pharmacy's initial submission of a clean claim during
28 which the pharmacy may initiate an appeal.

29 3. The pharmacy benefits manager shall respond to an appeal
30 within seven business days after the date on which the pharmacy
31 benefits manager receives the appeal.

32 *a.* If the pharmacy benefits manager grants a pharmacy's
33 appeal related to a reimbursement rate, the pharmacy benefits
34 manager shall do all of the following:

35 (1) Adjust the reimbursement rate of the prescription drug

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1 that is the subject of the appeal and provide the national drug
2 code number that the adjustment is based on to the appealing
3 pharmacy.

4 (2) Reverse and resubmit the claim that is the subject of
5 the appeal.

6 (3) Make the adjustment pursuant to subparagraph (1)
7 applicable to all of the following:

8 (a) Each pharmacy that is under common ownership with the
9 pharmacy that submitted the appeal.

10 (b) Each pharmacy in the state that demonstrates the
11 inability to purchase the prescription drug for less than the
12 established reimbursement rate.

13 b. If the pharmacy benefits manager denies a pharmacy's
14 appeal, the pharmacy benefits manager shall do all of the
15 following:

16 (1) Provide the appealing pharmacy the national drug
17 code number and the name of a wholesale distributor licensed
18 pursuant to section 155A.17 from which the pharmacy can obtain
19 the prescription drug at or below the reimbursement rate.

20 (2) If the prescription drug identified by the national
21 drug code number provided by the pharmacy benefits manager
22 pursuant to subparagraph (1) is not available below the
23 pharmacy acquisition cost from the wholesale distributor from
24 whom the pharmacy purchases the majority of its prescription
25 drugs for resale, the pharmacy benefits manager shall adjust
26 the reimbursement rate above the appealing pharmacy's pharmacy
27 acquisition cost, and reverse and resubmit each claim affected
28 by the pharmacy's inability to procure the prescription drug
29 at a cost that is equal to or less than the previously appealed
30 reimbursement rate.

31 Sec. 8. SEVERABILITY. The provisions of this division of
32 this Act are severable pursuant to section 4.12.

33 Sec. 9. APPLICABILITY. This division of this Act applies
34 to pharmacy benefits managers, health carriers, third-party
35 payors, and health benefit plans that manage a prescription

1 drug benefit in the state on or after July 1, 2025.

2 DIVISION II

3 PHARMACY SERVICES ADMINISTRATIVE ORGANIZATIONS AND WHOLESALE
4 DISTRIBUTION OF PRESCRIPTION DRUGS

5 Sec. 10. PHARMACY SERVICES ADMINISTRATIVE ORGANIZATIONS AND
6 WHOLESALE DISTRIBUTION OF PRESCRIPTION DRUGS — REPORT.

7 1. By January 1, 2026, the commissioner of insurance, or
8 the commissioner of insurance's designee, shall review pharmacy
9 services administrative organizations and the wholesale
10 distribution of prescription drugs, and submit a report to the
11 general assembly containing the commissioner's findings and
12 recommendations. The report shall include, at a minimum, all
13 of the following:

14 a. A description and analysis of the prescription drug
15 wholesale distribution supply chain, including the market
16 concentration for the wholesale distribution of prescription
17 drugs, margins in the wholesale distribution of prescription
18 drugs, and the competition in the wholesale distribution of
19 prescription drugs.

20 b. A description of the role that pharmacy services
21 administrative organizations serve in the prescription drug
22 supply chain.

23 c. A description and analysis of the relationships between
24 pharmacy services administrative organizations, prescription
25 drug wholesalers, and retail pharmacies, including but
26 not limited to standard contracting terms, fees charged to
27 pharmacies, and contractual restrictions and limitations
28 applicable to retail pharmacies.

29 2. a. The commissioner of insurance shall submit the report
30 under subsection 1 in a manner that does not publicly disclose
31 any of the following:

32 (1) The identity of a specific pharmacy services
33 administrative organization or prescription drug wholesaler.

34 (2) The price charged to a specific pharmacy for a specific
35 prescription drug.

1 b. Information provided by the commissioner under this
2 section that may reveal the identity of a specific pharmacy
3 services administrative organization or prescription drug
4 wholesaler, or the price charged to a specific pharmacy for a
5 specific prescription drug, shall be considered a confidential
6 record.>

7 2. Title page, by striking lines 1 and 2 and inserting
8 <An Act relating to pharmacy benefits managers, pharmacies,
9 prescription drugs, and pharmacy services administrative
10 organizations, and including applicability provisions.>

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